

Health Summary Report

Patient: ashig
DOB: None
Blood Type: None

Medical History

- Hypertension
- Type 2 Diabetes
- Hyperlipidemia (elevated LDL)

Insights from Documents (AI Synthesis)

****Comprehensive Health Assessment: ashig****

****Date of Assessment:**** March 13, 2026

****Patient Name:**** ashig

****Presenting Illness:****

ashig presents with a history of persistent sharp sub-sternal chest pain that radiates to the left arm. Episodes typically last approximately 15 minutes, are often triggered by physical exertion, and are reliably relieved by rest or sublingual nitroglycerin. This clinical picture is highly suggestive of anginal symptoms.

****Medical History:****

The patient has a documented history of hypertension, currently managed with Lisinopril 10mg, and Type 2 Diabetes, managed with Metformin.

****Recent Metrics (March 12, 2026):****

* ****Blood Pressure:**** Recent readings include 160 mmHg, 100 mmHg, and 118 mmHg. The 160/100 mmHg reading is concerning and indicates suboptimal control of the patient's known hypertension.

* ****Temperature:**** 45.0 °C. This is an critically elevated temperature and represents a significant anomaly. It is highly unlikely to be an accurate physiological reading and suggests a potential data entry error, as such a temperature is incompatible with sustained life.

* ****Weight:**** 70.0 kg.

****Lab Results:****

A recent lipid profile indicates an elevated Low-Density Lipoprotein (LDL) level of 160 mg/dL.

****Diagnosis:****

Suspected Stable Angina Pectoris.

****Clinical Assessment:****

ashiq is a patient with a significant cardiovascular risk profile, including established hypertension and Type 2 Diabetes, both of which are currently managed but not optimally controlled based on recent metrics. The patient's presenting symptoms are classic for stable angina pectoris, strongly supporting the suspected diagnosis.

The recent blood pressure reading of 160/100 mmHg highlights uncontrolled hypertension, which is a critical modifiable risk factor contributing to the patient's anginal symptoms and overall cardiovascular risk. Furthermore, the elevated LDL cholesterol level at 160 mg/dL indicates dyslipidemia, another major contributor to atherosclerotic disease progression. The reported temperature of 45.0 °C is a severe outlier and must be considered a data anomaly; if accurate, it would indicate an acute, life-threatening emergency, which is inconsistent with the patient's presentation of stable angina and the planned outpatient management.

The combination of typical anginal symptoms, uncontrolled hypertension, Type 2 Diabetes, and elevated LDL cholesterol places ashiq at a very high risk for ongoing myocardial ischemia and future adverse cardiovascular events. Prompt and comprehensive management of both the symptomatic angina and underlying comorbidities is essential.

****Recommended Next Steps:****

- **Cardiology Evaluation:**** Proceed with the planned Stress Thallium test to confirm the diagnosis of myocardial ischemia, assess its severity, and guide further management strategies.
- **Blood Pressure Management:**** Increase Lisinopril dosage as per the current plan. Close monitoring of blood pressure is crucial to achieve and maintain target levels (e.g., <130/80 mmHg for patients with diabetes and hypertension) to mitigate cardiovascular risk.
- **Dyslipidemia Management:**** Initiate or intensify statin therapy to aggressively lower LDL cholesterol, aiming for a target reduction appropriate for a high-risk patient (e.g., <70 mg/dL or a 50% reduction).
- **Temperature Verification:**** Critically re-verify the recorded temperature of 45.0 °C. If this was a true reading, it warrants immediate medical investigation, although it is highly suspected to be a data entry error.
- **Diabetes Control:**** Review and optimize current Type 2 Diabetes management. Assess recent HbA1c levels and adjust therapy as needed to ensure optimal glycemic control, which is vital for preventing micro- and macrovascular complications.
- **Lifestyle Modifications:**** Reinforce comprehensive lifestyle counseling, including a heart-healthy diet (low sodium, low saturated/trans fats), regular physical activity (as appropriate given anginal symptoms and guided by stress test results), and smoking cessation if applicable.
- **Symptomatic Management:**** Continue sublingual nitroglycerin for acute symptom relief and educate the patient on its proper use and when to seek emergency care.

Health Metrics & Vital Trends

Date	Metric	Value	Notes
2026-03-12	blood_pressure	160.0 mmHg	-
2026-03-12	blood_pressure	100.0 mmHg	-

2026-03-12	temperature	45.0 c	-
2026-03-12	weight	70.0 kg	-
2026-03-12	blood_pressure	118.0 mmHg	-

Recent Clinical Visits

Date	Diagnosis	Treatment
2026-03-08	heart attack	
2026-03-08	Possible Heart Attack, Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	CALL 108 IMMEDIATELY, Chew and swallow aspirin (if not allergic), Sit down and rest, Loosen tight clothing, Stay calm and breathe slowly, Do not drive yourself to hospital
2026-03-04	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
2026-03-04	Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	Seek emergency care immediately (call 108), Chew aspirin 325mg if not allergic and no contraindications, Rest in a comfortable semi-reclined position, Loosen tight clothing around the chest, Monitor pulse and breathing
2026-03-04	Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	Seek emergency care immediately (call 108), Chew aspirin 325mg if not allergic and no contraindications, Rest in a comfortable semi-reclined position, Loosen tight clothing around the chest, Monitor pulse and breathing
2026-03-04	Migraine, Tension-Type Headache, Hypertensive Headache	CALL 108 IMMEDIATELY - Time is critical, Note the time symptoms started, Do not give aspirin or other medications, Keep patient calm and lying down, Monitor breathing and consciousness
2026-03-03	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
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Report generated on: 2026-03-17T04:36:31.599688

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